Dry Nights Ahead

A Practical, Age-by-Age Guide to Understanding and Overcoming Bedwetting
From Toddlers to Young Adults (Ages 0–19)

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First edition.

Important medical disclaimer. This book is for general information and education only. It is not a substitute for professional medical advice, diagnosis, or treatment. Every child is different. Always talk to your child's doctor, pediatrician, nurse, or other qualified health professional about your child's specific situation, and before starting, stopping, or changing any medication. Never ignore professional advice or delay seeking it because of something you have read in this book. If your child has any of the warning signs listed in Chapter 3 — such as pain when urinating, excessive thirst, weight loss, blood in the urine, or wetting that suddenly returns after a long dry spell — contact a doctor promptly.

The information in this book reflects published clinical guidance and research available at the time of writing, including guidance from the International Children's Continence Society (ICCS), the UK National Institute for Health and Care Excellence (NICE), the American Academy of Pediatrics (AAP), and peer-reviewed studies. Medical

knowledge changes, and recommendations may differ between countries. The author and publisher accept no liability for any loss or harm arising from the use of this information.

Product types (alarms, mattress protectors, absorbent pants) are described generically. No brand has sponsored this book.

How to Use This Book

You can read this book cover to cover, but you don't have to.

- **Part One (Chapters 1–3)** explains *why* bedwetting happens, the proven tools that fix it, and the warning signs that mean you should see a doctor. Everyone should read these chapters — they make every later chapter make sense.
- **Part Two (Chapters 4–9)** is the heart of the book: one chapter for each age group. Go straight to your child's age. Each chapter ends with a **Clear Action Plan** — a step-by-step checklist you can follow tonight.
- **Part Three (Chapters 10–12)** covers the situations that come up for every family: sleepovers and camps, setbacks, emotional wellbeing, siblings, cleaning up, and special circumstances such as ADHD, autism, and diabetes.
- **The Appendices** contain printable tools: a bladder diary, a reward chart, an alarm progress log, a fluid guide, a doctor's-visit checklist, and a one-page quick reference for every age.

> **A note on the age groups.** This book uses six age groups: 0–2, 3–5, 6–8, 9–12, 13–16, and 16–19. Sixteen-year-olds sit on the border — read both Chapter 8 and Chapter 9 and use whichever fits your teen best. Ages are a guide, not a rule; a mature 12-year-old may get more from the teen chapter, and a young-for-their-age 6-year-old may benefit from parts of the 3–5 chapter.

> **A note on language.** "Bedwetting" and "nocturnal enuresis" (the medical term) mean the same thing: wetting during sleep.

> This book uses “your child” throughout, even in the teen chapters — and those chapters are written so you can hand them to your teenager directly.

Introduction: You Are Not Alone

If you are reading this book, chances are you've changed a lot of wet sheets. Maybe you've stood in a dark hallway at 2 a.m. with a crying child and an armful of laundry, wondering what you're doing wrong. Maybe you are a teenager yourself, quietly searching for answers because you're too embarrassed to ask anyone.

Here is the most important thing this book will tell you, so it goes first:

Bedwetting is common, it is nobody's fault, and it is treatable.

Roughly one in five five-year-olds wets the bed. Around one in ten seven-year-olds and about one in twenty ten-year-olds still do. Even among teenagers, around 1–3 in every 100 are still wetting at night — which means in a typical high school, there are likely dozens of students quietly dealing with exactly the same thing. They just don't talk about it.

Bedwetting is not laziness, defiance, or a sign of emotional damage. Children do not do it on purpose, and they cannot stop it by "trying harder." It happens because of how a child's body works during sleep: how much urine their kidneys make at night, how much their bladder can hold, and how easily their brain wakes up to the bladder's signal. These are things that mature with time — and, importantly, things that can be helped with the right approach.

For decades, families were told to "just wait — they'll grow out of it." Many children do. But waiting is not the only option, and for older children it is often not the best one. Bedwetting can take a real toll on self-esteem, friendships, family stress, and a child's willingness to go on school trips and sleepovers. Research shows that

children who are successfully treated tend to see measurable improvements in self-esteem. Modern treatments — especially the bedwetting alarm and the medication desmopressin — help the large majority of children, and simple changes to drinking, toileting, and bowel habits help many more.

This book brings together the current medical guidance and the practical, real-world know-how that families need. It tells you not only *what* to do, but *when* to do it and *how* to do it at each stage of growing up — because what's right for a two-year-old is completely different from what's right for a seventeen-year-old heading off to college.

Let's get started.

Part One: Understanding Bedwetting

Chapter 1: Understanding Bedwetting: What It Is and Why It Happens

What counts as “bedwetting”?

Doctors use the term **nocturnal enuresis** for wetting during sleep in a child aged five or older. Before five, night-time wetting is a normal part of development and is not considered a medical problem at all.

There are a few important subtypes. Knowing which one your child has helps you choose the right approach.

Type	What it means	Why it matters
Primary enuresis	The child has never been reliably dry at night for six months or more.	The most common type (about 75–80% of cases). Usually a developmental issue that responds well to standard treatment.
Secondary enuresis	Wetting returns after the child has been dry for at least six months.	Needs a doctor’s check, because a new cause (infection, constipation, diabetes, stress, sleep problems) may be behind it.
Monosymptomatic enuresis	Night wetting only, with no daytime bladder symptoms.	Usually responds to alarm or desmopressin.

Type	What it means	Why it matters
Non-monosymptomatic enuresis	Night wetting *plus* daytime symptoms — urgency, frequent peeing, daytime accidents, holding manoeuvres, weak stream.	The daytime bladder problem should be treated first or alongside; a doctor's assessment is recommended.

How common is it?

Bedwetting is one of the most common childhood conditions. Figures vary between studies and countries, but a consistent picture emerges:

Age	Approximate share of children who wet the bed
5 years	about 15–20% (1 in 5 to 1 in 7)
7 years	about 7–10%
10 years	about 5%
15 years	about 1–3%
Adults	about 0.5–2%

Boys are affected more often than girls in childhood (roughly twice as often), though the gap narrows in the teen years.

Without treatment, roughly **14–15% of children who wet the bed become dry each year on their own**. That sounds encouraging — and it is — but it also means that in any given year, about 85% of children who wet will still be wetting next year. Research also suggests that the more often a child wets (for example, every night rather than once a week) and the older they are, the less likely it is to clear up by itself. That is why older children, and those who wet most nights, benefit most from active treatment.

The three main causes

Almost all bedwetting comes down to a mismatch between three things. Think of it as a simple equation:

> **Bedwetting happens when the amount of urine made overnight is more than the bladder can hold — and the child doesn't wake up to go.**

1. Too much urine at night (nocturnal polyuria)

Normally, the brain releases more of a hormone called **vasopressin** (also called antidiuretic hormone, or ADH) at night. Vasopressin tells the kidneys to make less, more concentrated urine while we sleep. In many children who wet the bed, this night-time rise in vasopressin is smaller or delayed, so the kidneys keep producing daytime amounts of urine all night. This is the type of bedwetting that responds especially well to the medication **desmopressin**, which is a man-made version of vasopressin.

Large evening drinks, caffeine, fizzy drinks, and salty or high-protein late meals can also increase night-time urine.

2. A bladder that holds less at night (reduced bladder capacity or overactivity)

Some children have a bladder that holds less than expected for their age, or that contracts ("squeezes") before it is full. These children often also show daytime signs: needing to go urgently, going very often, little daytime accidents, or doing the "potty dance" — crossing legs, squatting, or pressing on their crotch. Constipation frequently makes this worse (see below). This type responds especially well to

the **bedwetting alarm**, to good daytime drinking and toileting habits, and to treating constipation.

3. Not waking up to the bladder's signal (arousal)

Most children who wet the bed are not unusually "deep" sleepers in the ordinary sense, but their brain does not respond to the signal of a full bladder by waking them up. This is a maturation issue in the connection between bladder and brain. It is not something a child can control by willpower. The bedwetting alarm works largely by training this connection.

Other factors that contribute

Genetics. Bedwetting runs strongly in families. Classic studies estimate that if neither parent wet the bed as a child, the risk is about 15%. If one parent did, the risk rises to about 44%; if both did, to about 77%. Many parents only discover that they — or a grandparent, aunt, or uncle — wet the bed as a child when they ask. Sharing this with your child can be enormously reassuring.

Constipation. This is one of the most overlooked causes. A rectum full of stool presses on the bladder, reducing how much it can hold and irritating it so it contracts unexpectedly. Constipation is much more common in children who wet the bed than in other children, and studies show that children who are constipated respond less well to bedwetting treatments until the constipation is treated. Importantly, a child can be constipated *even if they poo every day* — if the stools are hard, small, painful, or very large and infrequent. See Chapter 2 for how to spot and manage it.

Sleep-disordered breathing. Loud snoring, pauses in breathing, mouth breathing, and restless sleep can be signs of obstructed

breathing during sleep, often from large tonsils or adenoids. This can increase night-time urine production and disrupt normal waking. In some children, treating the airway problem resolves the bedwetting.

Neurodevelopmental conditions. Children with ADHD or autism have higher rates of bedwetting. They can still be treated successfully, though approaches may need adapting (see Chapter 12).

Stress and life events. Stress does not usually *cause* primary bedwetting, but it can trigger secondary bedwetting (a return of wetting after being dry) — for example after a new sibling, a house move, starting school, parental separation, bereavement, or bullying. Bedwetting itself also causes stress, which can create a vicious cycle.

Medical conditions (uncommon). A small number of cases are caused by urinary tract infection, diabetes (type 1 or type 2), kidney problems, structural problems of the urinary tract, or nerve problems affecting the spine. These usually come with other warning signs, listed in Chapter 3.

What does *not* cause bedwetting

It is worth saying clearly, because these myths still circulate:

- It is **not** laziness, attention-seeking, or bad behaviour.
- It is **not** caused by bad parenting.
- It is **not**, in most cases, a sign of emotional disturbance or trauma.
- It is **not** something a child can stop by trying harder or being punished.

Bedwetting myths and facts

Myth	Fact
“They’ll definitely grow out of it.”	Most will, eventually — but about 1–2% of adults still wet. Waiting years is not necessary when effective treatments exist.
“Cutting out all drinks after lunch will fix it.”	Restricting fluids too much can *reduce* bladder capacity and cause constipation. Children need to drink well during the day; only the last hour before bed should be limited.
“Waking them at night to pee will train them.”	“Lifting” or waking at set times can reduce wet beds in the short term but does not teach long-term dryness (NICE guidance). It’s a practical measure, not a cure.
“Punishing or shaming will motivate them.”	Punishment makes bedwetting worse and harms self-esteem. It should never be used.
“Pull-ups make bedwetting last longer.”	Absorbent pants don’t cause bedwetting. For children starting an alarm program, though, they are usually removed so the alarm can detect wetting.
“Bedwetting means something is psychologically wrong.”	In most children it is a physical, developmental issue. Emotional difficulties are more often a *result* of bedwetting than a cause.

The good news: it can be treated

Today’s main treatments are effective for most children:

- **Bedwetting alarms** help around two-thirds or more of children who use them consistently, and they have lower relapse rates than medication once treatment ends.
- **Desmopressin** reduces wetting in most children while they take it and works quickly — often within days — making it ideal for sleepovers and camps.
- **Basic measures** — good daytime drinking, regular toilet visits, treating constipation, and positive encouragement — help many children on their own and make the other treatments work better.

The next chapter explains each of these tools in detail.

Chapter 2: The Bedwetting Toolkit: Proven Strategies and Treatments

This chapter describes the full set of tools available. The age-group chapters in Part Two tell you *which* tools to use, *when*, and *how* for each stage. Come back to this chapter whenever you need the details.

Tool 1: Healthy drinking habits

Children who wet the bed often drink too little during the day and too much in the evening — sometimes because they (or their parents) are trying to avoid wetting. This pattern makes things worse. The goal is to **drink well and evenly during the day, and taper off before bed**.

Daily fluid guide (water-based drinks, approximate). These ranges are based on NICE guidance and vary with weather, activity, and diet:

Age	Girls	Boys
4–8 years	1,000–1,400 mL (about 4–6 cups)	1,000–1,400 mL (about 4–6 cups)
9–13 years	1,200–2,100 mL (about 5–9 cups)	1,400–2,300 mL (about 6–10 cups)
14–18 years	1,400–2,500 mL (about 6–10 cups)	2,100–3,200 mL (about 9–13 cups)

(One cup ≈ 240 mL / 8 fl oz.)

How to spread drinks across the day:

- Aim for **6–8 drinks spread through the day**: on waking, with breakfast, mid-morning, lunch, mid-afternoon, after school,

and with the evening meal.

- Try to have **most of the day's fluid before about 5 p.m.** — a full water bottle at school helps enormously. Check that your child's school allows easy access to water and the toilet.
- Allow a small drink with the evening meal, then **stop drinks about one hour before bed** (provided they've drunk well during the day).
- **Avoid caffeine** — cola, energy drinks, tea, coffee, and chocolate drinks — especially in the afternoon and evening. Caffeine increases urine production and irritates the bladder.
- **Limit fizzy (carbonated) drinks and sugary drinks**, which can also irritate the bladder.
- Water is best. Milk is fine at meals.
- Be mindful of **salty, high-protein late snacks**, which can increase night-time urine.

Tool 2: Regular toilet habits

- **Daytime:** Encourage your child to go to the toilet regularly — about **every 2–3 hours, roughly 4–7 times a day** — rather than holding on until the last minute.
- **Before bed:** Use the "double void": go to the toilet as part of the bedtime routine, then go again just before lights out.
- **Posture matters:** Children should sit comfortably with feet supported (use a footstool if their feet don't touch the floor) and knees slightly apart, relaxed, and take their time. Boys who tend to rush may benefit from sitting to pee in the evening to empty fully.
- **Make night-time access easy:** Night-lights in the hallway and bathroom, a clear path, and — if needed — a potty or portable urinal in the bedroom.

Tool 3: Treating constipation

Because constipation is so closely linked to bedwetting, checking for it is one of the most valuable things you can do.

Signs your child may be constipated:

- Fewer than three poos a week, **or**
- Hard, lumpy, or pellet-like stools (types 1–2 on the Bristol Stool Chart), **or**
- Straining, pain, or bleeding when pooing, **or**
- Very large stools that may block the toilet, **or**
- Soiling or skid-marks in underwear, **or**
- Tummy pain, poor appetite, or avoiding the toilet.

What helps:

- Plenty of fluids (see Tool 1) and fibre — fruit, vegetables, whole grains, beans, lentils.
- Regular, relaxed toilet sits for 5–10 minutes after meals (when the natural gut reflex is strongest), with feet supported on a stool.
- Daily physical activity.
- If constipation persists, **see a doctor**. Doctors commonly recommend an osmotic laxative (such as polyethylene glycol / macrogol), often for weeks or months. Don’t stop laxatives suddenly once things improve — follow your doctor’s plan.

Treating constipation can resolve bedwetting entirely in some children and makes alarms and medication work better in others.

Tool 4: Positive reinforcement and reward systems

Rewards can help motivate children — but only if they reward things the child can control. **Never reward dry nights alone**, because a child can't control those directly, and failing to earn the reward feels like punishment.

Do reward:

- Drinking the recommended amount during the day.
- Going to the toilet before bed (and doing the "double void").
- Getting up to use the toilet at night.
- Waking up to the alarm and helping to reset it.
- Helping to change the sheets without fuss.

How:

- Use a simple chart with stickers or stars (see Appendix B).
- Agree small, frequent rewards (a sticker, extra story, choosing dinner) and occasional bigger ones for a run of stars.
- Praise specifically: "You got up to the toilet all by yourself — that's brilliant."
- Keep it fun. If the chart becomes stressful, stop and try again later.

Reward systems on their own suit younger children (roughly 5–7 years) with infrequent wetting. For older children and frequent wetting, they work best combined with an alarm or medication.

Tool 5: The bedwetting alarm

The bedwetting alarm is the most effective long-term treatment for most children and is recommended as **first-line treatment** by both

NICE and the International Children's Continence Society (ICCS) for motivated families.

How it works

A small moisture sensor (in the underwear, on a pad under the sheet, or in a clip) detects the first drops of urine and triggers an alarm (sound, vibration, light, or a combination). At first, the alarm wakes the child (or more often, the parent) *after* wetting has started. Over weeks, the brain learns to link the feeling of a full bladder with waking up — and eventually, many children start either waking before they wet, or sleeping through the night without needing to go, because the bladder learns to "hold on."

Types of alarm

Type	How it works	Good for
Body-worn (wearable)	Sensor clips to underwear; alarm unit clips to pyjamas near the shoulder.	Most children; detects wetting fastest.
Bell-and-pad (mat)	Sensor mat goes under the sheet; alarm sits beside the bed.	Children who dislike anything attached to them; heavy sleepers.
Wireless	Sensor sends a signal to a receiver in the child's room and/or the parent's room.	Families where a parent needs to hear the alarm from another room.
Vibrating	Vibrates instead of / as well as sounds.	Children with hearing impairment; teens sharing rooms; dormitories.

Bedwetting alarms are available to buy and, in some countries, may be loaned through health services or continence clinics — ask your doctor or nurse.

How to use an alarm successfully

1. **Choose the right time.** Start when your family has a stable few months — not during exams, a house move, a new baby, or holidays.
2. **Commit to nightly use.** The alarm must be used *every night*. Inconsistent use is the most common reason alarms fail.
3. **Explain it to your child** positively, as a training tool — like training wheels for the brain.
4. **Practise during the day.** Have your child lie in bed, "pretend" the alarm sounds (trigger it with a damp cloth), and act out getting up, switching it off, going to the toilet, and coming back. Do this a few times.
5. **No absorbent pants** — the sensor must detect wetness. Use a waterproof mattress protector.
6. **When the alarm goes off:**
    - The child (with your help at first) should wake up fully, switch off the alarm, and go to the toilet to finish peeing.
    - Change wet clothes and bedding together, calmly. Keep spare pyjamas and a towel ready.
    - Reset the alarm before going back to sleep.
    - Many children don't wake to the alarm at first. **A parent needs to wake the child** — calmly, using their name, sitting them up, until they are truly awake. This is normal and usually needed for the first few weeks.
7. **Keep a record** of wet and dry nights and the size of the wet patch (see Appendix C). Shrinking wet patches and the alarm going off later in the night are early signs of progress.

8. **Give it time.** Assess progress after about **4 weeks**. NICE suggests continuing if there are signs of improvement, and it often takes **6–16 weeks** to achieve dryness. In one large study, the average time to dryness was about two months.
9. **Know when it's done.** Continue until your child has achieved **14 consecutive dry nights**.
10. **Optional — "overlearning" to prevent relapse:** After 14 dry nights, some clinicians suggest giving an extra drink (about 200–500 mL, depending on age) an hour before bed while continuing the alarm, until 14 more consecutive dry nights are achieved. This can strengthen the habit. Discuss with your health professional before trying this.

What to expect

- Studies report that alarm treatment leads to dryness in roughly **50–80% of children** who use it properly, with one large clinical series reporting 76% success.
- Some children relapse after stopping (estimates range widely). If wetting returns, **restarting the alarm** usually works faster the second time.
- Alarms are disruptive for the whole household at first. Planning, team spirit, and a sense of humour help.

When an alarm may not suit your family

NICE notes an alarm may be unsuitable if wetting is very infrequent (fewer than 1–2 times a week), if parents are under significant stress or are struggling emotionally with the bedwetting, or if the child or family is not motivated. In those cases, desmopressin is usually the alternative.

Tool 6: Desmopressin (medication)

Desmopressin is a synthetic version of the body's natural antidiuretic hormone (vasopressin). Taken at bedtime, it reduces the amount of urine the kidneys produce overnight.

- It is a **first-line treatment** alongside the alarm (ICCS, NICE), and is especially useful when:
  - fast or short-term improvement is the priority (sleepovers, camps, school trips);
  - the alarm isn't suitable or hasn't worked;
  - the child produces large volumes of urine at night (nocturnal polyuria).
- It is available as **tablets** or **melt-in-the-mouth tablets (melts)**, prescribed by a doctor. It is generally used in children aged 5 and over.
- It works quickly — often within the first few nights — and helps the majority of children reduce wetting, with a good proportion becoming completely dry while taking it.
- It does **not cure** the underlying cause. Many children start wetting again when it is stopped, so doctors often plan structured breaks (e.g., a week off every three months) to check whether it is still needed, and may reduce the dose gradually rather than stopping suddenly.

Essential safety rules for desmopressin

Desmopressin is safe when used correctly, but **drinking too much while taking it can cause a dangerous drop in blood sodium (hyponatremia)**, which in rare cases causes seizures.

- **Limit drinks from 1 hour before taking the dose until 8 hours afterward** — only small sips if thirsty. Follow your

doctor's exact instructions.

- **Stop taking it during any illness that causes vomiting, diarrhoea, or fever,** or whenever your child may need to drink a lot (e.g., hot weather or heavy exercise) — and restart only when well.
- **Seek medical advice immediately** if your child has a headache, nausea, vomiting, confusion, or unusual drowsiness while taking it.
- The **nasal spray** form is **no longer approved for bedwetting** in the US and UK because of a higher risk of low sodium. Tablets or melts are used instead.
- Always use the dose prescribed and never give extra.

Tool 7: Combination and specialist treatments

If the alarm or desmopressin alone doesn't work, a doctor or specialist continence clinic may suggest:

- **Alarm plus desmopressin** together.
- **Anticholinergic medications** (e.g., oxybutynin), which calm an overactive bladder, usually combined with desmopressin, especially when there are daytime symptoms. Constipation must be managed first because these medicines can worsen it.
- **Imipramine** (a tricyclic medication) — a specialist-only option used rarely because of potential side effects and danger in overdose.
- **Bladder training programs, pelvic floor work, or urotherapy** with a specialist nurse or physiotherapist, especially for children with daytime symptoms.
- **Investigation of the airway** (ear, nose and throat assessment) if snoring and breathing pauses suggest sleep-disordered breathing.

- **Psychological support** where bedwetting is causing significant distress, low self-esteem, or behaviour problems, or where stress is a trigger.

Tool 8: Practical protection (managing, not treating)

These don't cure bedwetting, but they make life much easier and protect dignity:

- **Waterproof mattress protector** and pillow protector (breathable, quiet types are best for comfort).
- **Layering the bed** ("double-making"): protector – sheet – protector – sheet. If the bed gets wet at night, strip the top two layers and there's a dry bed underneath — no fumbling at 2 a.m.
- **Absorbent bed pads** placed on top of the sheet.
- **Absorbent pants / pull-ups / disposable or washable night pants** in age-appropriate sizes, for children not on an alarm program, for travel, or for sleepovers. Discreet teen and young-adult versions exist.
- **A "night kit"** by the bed: clean pyjamas, underwear, wipes, a towel, and a bag for wet items, so your child can change independently.

Things that don't work (or are not recommended)

- **Punishment, shaming, or teasing** — harmful and makes wetting worse.
- **Severe fluid restriction** throughout the day.
- **"Lifting" or waking as a long-term strategy** — fine as a short-term practical measure, but it doesn't build long-term dryness.

- **Unproven products** promising “instant cures,” herbal remedies, or devices without evidence. Some complementary approaches (such as acupuncture or hypnotherapy) have limited evidence and are not recommended as standard care; if you want to try them, discuss them with your doctor and don’t let them replace proven treatment.

Chapter 3: When to See a Doctor: Red Flags and What to Expect

Routine reasons to see a doctor

You should talk to a doctor, pediatrician, or nurse about bedwetting if:

- Your child is **5 years or older** and still wetting the bed regularly and it is bothering your child or your family.
- Your child is **7 or older** and still wetting — treatment is generally recommended by this age.
- Your child has **daytime symptoms** too: daytime accidents, urgency, going very often, or holding manoeuvres.
- Your child seems **constipated** or has soiling.
- The bedwetting is causing **distress**, low self-esteem, bullying, or is stopping your child from joining in activities.
- You have tried basic measures for a few months without improvement.

You do *not* need to wait for a certain age if your child is upset by it. NICE guidance specifically states that children under 5 should not be excluded from management if their bedwetting is a concern.

Red flags: see a doctor promptly

Seek medical advice **soon** (within days) if bedwetting comes with any of these:

- **Wetting that starts again after six months or more of being dry** (secondary enuresis), especially if sudden.

- **Excessive thirst, drinking much more than usual, needing to pee much more often, weight loss, or tiredness** — these can be signs of **diabetes**, which needs urgent testing.
- **Pain, burning, or stinging when peeing, cloudy or smelly urine, fever, or blood in the urine** — possible **urinary tract infection**.
- **A weak or dribbling urine stream, straining to pee**, or being constantly damp.
- **Loud snoring, gasping, or pauses in breathing during sleep**.
- **Leg weakness, changes in walking, back pain, or a dimple, tuft of hair, or lump at the base of the spine** — possible nerve problems.
- **Soiling, severe constipation, or tummy swelling**.
- **Any concern about abuse or significant emotional trauma** — sudden-onset wetting together with changes in behaviour should always be taken seriously.
- **Wetting in a teenager or young adult that is new or getting worse.**

> **Go to an emergency department or call emergency services** if your child is very unwell, drowsy, vomiting, breathing fast, or has severe tummy pain alongside excessive thirst and urination (possible diabetic ketoacidosis), or has a seizure, severe headache, or confusion while taking desmopressin.

Preparing for the appointment

Doctors find it enormously helpful when families arrive prepared. Before your visit:

1. **Keep a bladder diary for 2 days** (preferably a weekend) — record drinks, times and volumes of urine passed, and any accidents. See Appendix A.
2. **Record wet and dry nights for 2 weeks**, including whether wetting happened once or more, and roughly when.
3. **Note bowel habits** for a week — frequency and stool type.
4. **List any other symptoms**: snoring, daytime wetting, urgency, thirst, behaviour or sleep changes.
5. **Think about family history**: did parents, siblings, or other relatives wet the bed?
6. **Note what you've tried** and what happened.
7. **Write down your questions** (see Appendix E).

What the doctor will usually do

- **Take a history** — the type of wetting, daytime symptoms, drinking, bowels, sleep, development, family history, and how the family is coping.
- **Do a physical exam** — checking the tummy, the lower back and spine, the genitals, and sometimes the legs and reflexes.
- **Test the urine** (dipstick) — mainly to check for sugar (diabetes) and infection.
- **Most children need no further tests.** Blood tests, ultrasound scans, or specialist referral are only needed if something unusual is found or treatment doesn't work.
- **Agree a plan** — usually basic advice plus an alarm or desmopressin, and a follow-up appointment.

Remember: a good health professional will treat your child's bedwetting with respect, will talk *to* your child (not just about them), and will never blame you or your child.

Part Two: Age-by-Age Action Plans

Chapter 4: Ages 0–2: Laying the Foundations

What's normal at this age

At this age, **wetting at night is not bedwetting — it is simply normal**. Babies and toddlers do not have the physical ability to stay dry through the night. Their bladders are small, their bodies have not yet developed the night-time pattern of the hormone that reduces urine production, and the connection between bladder and brain that allows them to wake (or hold on) is still developing.

Most children gain **daytime** bladder control somewhere between about 18 months and 3 years. **Night-time dryness usually comes later** — commonly between 3 and 5 years, and sometimes not until 6 or 7. A small number of toddlers become dry at night early, often at the same time as daytime training, but most do not. Neither is "better."

So there is nothing to *treat* in this age group. But there is a great deal you can do to lay healthy foundations that make night-time dryness easier later — and to avoid creating problems (like constipation or toilet fear) that can make bedwetting more likely and longer-lasting.

What parents worry about at this age

- **"My 2-year-old is out of diapers in the day — why not at night?"** Day and night control are controlled differently. Daytime control depends on awareness and choice; night-time control depends on hormones, bladder size, and brain

maturation during sleep, which no amount of practice can rush.

- **“Should I stop night-time milk or drinks?”** Toddlers still need adequate fluid. Don’t restrict drinks to try to achieve dryness at this age.
- **“Should I wake my toddler to pee?”** There’s no need, and it can disrupt everyone’s sleep.

The goals for ages 0–2

1. Support **healthy bladder and bowel development**.
2. **Prevent constipation**, which is one of the biggest avoidable contributors to later wetting.
3. Build a **positive, relaxed relationship with the toilet**.
4. Begin **daytime toilet training when your child is ready** — without pressure.
5. **Leave night-time dryness alone** for now.

Healthy foundations

Drinks

- Offer water regularly through the day, especially once your toddler is eating solids and in warm weather.
- After 12 months, cow’s milk (or an appropriate alternative) is usually offered at meals and snacks — follow your health professional’s guidance on amounts. Too much milk can reduce appetite for fibre-rich foods and contribute to constipation.
- Avoid sugary drinks, fizzy drinks, and anything with caffeine (including tea, cola, and chocolate drinks).

- Begin establishing the habit of **drinks with meals and between meals**, rather than lots of fluid in the evening.

Bowels

- Offer fibre-rich foods: fruits (pears, prunes, plums, berries), vegetables, whole grains, beans, and lentils.
- Watch for **signs of constipation**: hard, pellet-like stools; straining with pain; fewer than three poos a week; blood on the stool or diaper; or a toddler who hides, cries, or "holds on" when they need to poo.
- If your child is constipated, talk to your health visitor, pediatrician, or family doctor — **early treatment prevents a painful cycle of withholding** that can last for years and is linked to wetting.

Sleep

- A consistent, calming bedtime routine (bath, pyjamas, teeth, story, sleep) supports healthy sleep, which is good for every aspect of development.
- If your toddler **snores loudly most nights**, gasps, or has pauses in breathing, mention it to a doctor — enlarged tonsils and adenoids are common at this age and can affect sleep quality.

Daytime toilet training: readiness first

Night dryness comes after daytime control, so successful, *relaxed* daytime training is the foundation. Look for **signs of readiness**, which typically appear between about 18 months and 3 years:

- Stays dry for 1–2 hours at a time during the day.

- Shows awareness of wetting or soiling — tells you, pulls at the diaper, hides to poo.
- Shows interest in the toilet or in copying others.
- Can follow simple instructions.
- Can walk to the potty, sit, and get up.
- Can help pull pants up and down.

If your child is not showing these signs, wait. Starting too early tends to make training take longer, not shorter.

How to support daytime training

- **Get the equipment ready:** a potty and/or a toilet seat insert with a footstool, so feet are supported (this helps children relax their pelvic muscles and empty fully).
- **Introduce it playfully:** read toddler books about using the potty, let your child sit fully clothed, let them see family members use the toilet.
- **Build in regular sits:** on waking, after meals, before and after naps, before bath, and before bed. Sitting after meals takes advantage of the body's natural reflex to empty the bowel.
- **Praise effort, not results.** "You sat on the potty — great job!"
- **Expect accidents.** Respond calmly: "Oops, the pee came out. Let's get clean pants. Next time we'll try the potty."
- **Never punish, scold, or show disgust** — this can create toilet fear and withholding.
- **Don't force sitting** or keep a child on the potty for long periods.

What to do at night

- **Keep using diapers (nappies) or night-time pants at night.** This is not "going backward" — it's appropriate.

- **Don’t wake your child to pee** at this age.
- **Don’t restrict fluids** to try to achieve dryness.
- **Notice dry diapers.** If your child begins waking with a dry diaper on most mornings for a week or two, they may be ready to try nights without one — usually after their third birthday (see Chapter 5).

When to talk to a health professional

At this age, consult your doctor or health visitor if:

- Your child shows **signs of constipation** or pain when pooing.
- Your child seems to be **always wet** — constantly dribbling rather than wetting in “batches.” This can occasionally signal a structural issue.
- The **urine stream is weak** or your child strains to pee.
- Your child has **fevers without an obvious cause** (possible urinary tract infection — urine should be checked).
- Your child is **unusually thirsty**, drinking and urinating excessively, or losing weight.
- Your toddler **snores loudly** with breathing pauses.
- You are concerned about your child’s **development** more generally.

✅ Clear Action Plan: Ages 0–2

Do now

- ☐ Offer water regularly throughout the day; avoid sugary, fizzy, and caffeinated drinks.
- ☐ Include fibre-rich foods at every meal.

- [ ] Watch for constipation; see a health professional early if you spot it.
- [ ] Keep a calm, consistent bedtime routine.
- [ ] Keep diapers/night pants on at night — night dryness will come later.

When your child shows readiness signs (often 18 months – 3 years)

- [ ] Buy a potty or toilet insert and a footstool.
- [ ] Introduce the potty through play and books.
- [ ] Start regular, relaxed potty sits: on waking, after meals, before bath and bed.
- [ ] Praise every attempt; handle accidents calmly.

Don’t

- × Don’t restrict drinks or wake your child at night to try for dryness.
- × Don’t punish accidents or force sitting.
- × Don’t compare your child with others — the age range for dryness is wide.

See a professional if: constipation, constant dribbling, weak stream, unexplained fevers, excessive thirst, or loud snoring with pauses.

Chapter 5: Ages 3–5: The Journey to Dry Nights

What's normal at this age

This is the age when most children *begin* to become dry at night — but many are not there yet, and that is entirely normal. Around **one in five five-year-olds still wets the bed**. Doctors do not formally diagnose bedwetting until age 5, and many clinicians consider wetting up to age 6 or 7 part of the normal range.

Children in this group often fall into one of three patterns:

1. **Dry during the day and starting to wake up with dry diapers** — likely ready for nights without them.
2. **Dry during the day but consistently wet every morning** — the body isn't ready yet for night dryness.
3. **Still having frequent daytime accidents** — focus on daytime control first.

The goals for ages 3–5

1. **Consolidate daytime bladder and bowel habits.**
2. **Recognise readiness for night-time dryness** — and try it at the right time.
3. **Protect self-esteem** and keep things low-pressure.
4. **Identify any problems** early (constipation, daytime symptoms, snoring) — and, from age 5, consider whether a doctor's assessment would help.

Is my child ready for dry nights?

Signs that your child may be ready to go without night diapers or pants:

- **Dry diapers in the morning** on most days for at least a week or two.
- **Dry after daytime naps.**
- Reliably **dry during the day**, with few accidents.
- **Waking in the night** asking to go to the toilet.
- **Wanting** to wear “big kid” underwear at night.

If your child is waking wet every morning, they’re probably not ready — keep using night pants and check again every couple of months. There’s no harm in waiting.

How to start night training

1. **Pick a calm time** — avoid starting during illness, a house move, a new sibling’s arrival, or starting a new nursery or school.
2. **Talk about it positively.** “Your body is getting so good at holding pee. Shall we try sleeping in pants like a big kid?”
3. **Protect the bed:** waterproof mattress protector and the “double-make” method (protector – sheet – protector – sheet) for easy night changes.
4. **Bedtime routine:** a small drink with dinner, no big drinks in the last hour before bed, and a toilet visit as the last thing before lights out (and a “double void” — go once at the start of the routine, again at the end).
5. **Make night toilet trips easy:** a night-light in the bedroom, hallway, and bathroom; a potty in the bedroom if the toilet is

far away. Tell your child, “If you wake up and need to pee, you can go to the potty — or call me and I’ll help.”

6. **Give it a fair trial** — about **two to four weeks**.
7. **If there are frequent wet nights** (for example, most nights) after a few weeks, it’s perfectly okay to **go back to night pants** for a few months and try again. Present it positively: “Your body isn’t quite ready yet. We’ll try again after your birthday.”

Daytime habits that build night-time dryness

What happens during the day strongly influences what happens at night.

- **Drinks:** about 6–8 cups/drinks spread through the day (around 1,000–1,400 mL for ages 4–8), mostly before late afternoon. Water and milk are best. No caffeine; limit fizzy and sugary drinks.
- **Regular toilet visits:** encourage your child to go every 2–3 hours — on waking, at mid-morning, at lunch, mid-afternoon, after nursery or school, before dinner, and at bedtime. Young children often “forget” because they’re busy playing.
- **No holding on:** watch for the “potty dance” (crossing legs, wiggling, squatting on a heel). If you see it, gently suggest a toilet trip. Frequent holding can signal an overactive bladder — mention it to your doctor.
- **Healthy bowels:** fibre, fluids, activity, and relaxed toilet sits after meals with feet supported. Constipation at this age is common and very treatable.
- **Nursery/school:** let staff know your child needs easy access to water and the toilet.

Handling wet nights

- **Stay calm and matter-of-fact.** “Oops, the pee came out while you were sleeping. Let’s get you dry.” No sighing, no frustration in your voice.
- **Keep night changes quick and quiet**: dim light, minimal talking, fresh pyjamas, strip the top layer of the double-made bed, back to sleep.
- **Involve your child gently** — e.g., putting wet pyjamas in the laundry basket in the morning — as a normal routine, *never as a punishment*.
- **Wash your child in the morning** (a quick shower or bath, or wipes) to prevent skin irritation and odour.
- **Never punish, shame, or tease**, and don’t let siblings tease either.
- **Protect privacy**: don’t discuss wet nights in front of others.

Using rewards at this age

Simple sticker charts can work well for 4- and 5-year-olds — but reward the right things:

- ⭐ Drinking their water bottle during the day.
- ⭐ Going to the toilet before bed.
- ⭐ Getting up at night to use the toilet or potty.
- ⭐ Helping to put sheets in the laundry.

Avoid charts that reward dry nights alone — your child can’t control them yet, and missing out feels like failure.

Should we try “lifting” (waking to pee)?

Some families carry a sleeping child to the toilet before the parents go to bed. This can reduce the number of wet beds and may be a helpful short-term measure for some families, but research and NICE guidance indicate that it **does not teach long-term dryness**. If you use it, try to make sure your child is properly awake while using the toilet, and vary the time. Don’t feel you must do it.

When to see a doctor (ages 3–5)

Talk to your doctor or health professional if:

- Your child is **5 or older** and wetting at night is bothering them or the family.
- Your child has **daytime wetting** after age 4, urgency, very frequent peeing, or holding manoeuvres.
- Your child shows **signs of constipation** or soiling.
- Your child was **dry for six months or more and has started wetting again**.
- Your child **snores loudly** most nights or has breathing pauses.
- There are any **red-flag symptoms**: excessive thirst, weight loss, pain or burning when peeing, blood in urine, fevers, weak stream or constant dampness.

Treatments at this age

Bedwetting alarms and desmopressin are generally used from **age 5 upward**. Treatment is rarely needed before 5, but NICE guidance makes clear that **children under 5 should not be excluded** if wetting is causing problems — a health professional can advise on what’s appropriate. In most children aged 3–5, the focus is on daytime habits, constipation, and patience.

✅ Clear Action Plan: Ages 3–5

Every day

- ☐ 6–8 drinks spread across the day, mostly before late afternoon; no caffeine.
- ☐ Toilet visits every 2–3 hours and before bed (double void at bedtime).
- ☐ Fibre-rich foods; check poos for signs of constipation.
- ☐ Night-lights on the route to the toilet; potty in the bedroom if needed.

Deciding on night training

- ☐ Watch for dry mornings and dry naps for 1–2 weeks.
- ☐ If ready: protect the bed (double-make), talk positively, and try for 2–4 weeks.
- ☐ If mostly wet after 2–4 weeks: return to night pants without fuss and retry in 2–3 months.

Wet nights

- ☐ Stay calm and kind; quick, quiet changes.
- ☐ Wash in the morning; protect privacy; no teasing from siblings.
- ☐ Use a sticker chart for *actions* (drinking, toilet before bed, getting up), not for dry nights.

Book a doctor's visit if:

- ☐ Your child is 5+ and wetting is bothering them or you.
- ☐ Daytime symptoms, constipation, snoring, or any red flags (see Chapter 3).
- ☐ Wetting returned after 6+ months dry.

Chapter 6: Ages 6–8: Building Skills and Confidence

What's happening at this age

Between 6 and 8, roughly **7–10% of children still wet the bed** — in an average classroom of 30, that's two or three children. At this age bedwetting is now considered a medical condition (nocturnal enuresis) that **deserves active help**, not just waiting. Most experts and guidelines recommend offering treatment to children who are still wetting at this age, especially if they wet frequently or are bothered by it.

This is also the age when children become **much more aware** of bedwetting. They start going to friends' houses, sleepovers come up, school trips begin, and they compare themselves to peers. Feelings of embarrassment and shame can begin here. Your job is both to *treat* the bedwetting and to *protect* your child's confidence.

The good news: children aged 6–8 are often the **ideal age for a bedwetting alarm**. They're old enough to understand and participate, and motivated enough to want to be dry.

The goals for ages 6–8

1. **See a doctor** for an assessment if you haven't already.
2. **Optimise daytime habits**: drinking, toileting, bowels.
3. **Start active treatment**, usually a bedwetting alarm (or desmopressin if an alarm doesn't suit).

4. **Protect self-esteem** and help your child participate fully in social life.
5. **Involve your child** as an active partner.

Step 1: Talk to your child

Children of this age often worry silently. Open the conversation gently:

- **Normalise it:** “Lots of kids your age wet the bed. In your class there are probably a couple of others — they just don’t talk about it.”
- **Explain it simply:** “Your body makes pee at night, and your bladder holds it like a balloon. Your brain is supposed to wake you up when the balloon is full, but your brain is still learning that message. It’s not your fault, and it’s not because you’re lazy.”
- **Share family history** if there is one: “Did you know Dad wet the bed until he was 9? It runs in families.”
- **Make it a team project:** “We can do some things together to help your body learn. Want to be on the team?”
- **Ask how they feel** and what worries them (sleepovers? friends finding out?).

Step 2: See the doctor

Book an appointment, ideally after keeping the 2-day bladder diary and 2-week wet-night record described in Chapter 3 and Appendix A. The doctor will:

- Check for constipation, daytime bladder problems, and red flags.
- Test the urine.

- Discuss treatment options with you **and your child**.

Step 3: Optimise daytime habits (2–4 weeks)

Many children improve with these alone, and they make the alarm or medication more effective.

- **Drinks:** about 1,000–1,400 mL (around 6–8 cups) of water-based drinks a day, most before about 5 p.m. Send a labelled water bottle to school, and agree a target (e.g., “finish and refill once”).
- **Stop drinks about an hour before bed.** A small drink with dinner is fine.
- **No caffeine**: cola, iced tea, energy drinks, chocolate drinks.
- **Toilet visits**: on waking, at every school break, after school, before dinner, and twice at bedtime (double void). Teachers can help by reminding your child.
- **Bowels**: daily toilet sit after breakfast or dinner, with feet on a footstool. Look for hard stools, straining, or soiling, and ask your doctor about treatment if needed.
- **School:** ask teachers to allow free access to water and the toilet. Some schools discourage toilet trips in class — a short note from you or your doctor can help.

Step 4: Start treatment

Option A: The bedwetting alarm (first choice for most children)

The alarm is especially well suited to this age group. Follow the full instructions in Chapter 2. Key points for 6–8-year-olds:

- **Let your child help choose** the alarm (sound, colour, wearable or mat) — ownership boosts motivation.
- **Practise during the day**, turning it into a fun drill: lie in bed, the alarm "goes off," your child jumps up, switches it off, walks to the toilet, and comes back. Repeat a few times. Rehearsal really does help.
- **Expect to wake your child at first.** Most children this age sleep through the alarm in the early weeks. A parent may need to sleep within earshot (or use a wireless receiver) and wake the child fully — sit them up, call their name, maybe a cool damp cloth on the face — before walking them to the toilet.
- **Keep a progress chart** (Appendix C). Celebrate signs of progress: a smaller wet patch, the alarm going off later in the night, your child waking on their own, waking *before* the alarm.
- **Reward actions**: waking to the alarm, getting up to the toilet, helping reset the bed. Add a special treat for every few stars.
- **Persist.** Review progress after 4 weeks. If improving, keep going until **14 consecutive dry nights**. Typical time is 6–16 weeks.
- **If no improvement after about 4–8 weeks**, talk to your doctor about switching to or adding desmopressin.

Option B: Desmopressin

Your doctor may suggest desmopressin if:

- an alarm isn't suitable for your family;
- the alarm hasn't worked;
- your child wets very large amounts, often early in the night (suggesting high night-time urine production);
- quick results are needed (e.g., for a school trip).

Safety first: no drinks from one hour before the dose until the next morning (8 hours), and stop it if your child is vomiting or has diarrhoea. See the full safety rules in Chapter 2.

Option C: Combination

If neither works alone, the alarm and desmopressin can be used together, or your doctor may refer you to a specialist continence clinic.

Step 5: Protect confidence and social life

- **Sleepovers:** don't ban them. Instead, see Chapter 10 for a detailed plan: discreet absorbent pants, a sleeping bag with a liner, desmopressin if prescribed, and a trusted adult at the host home.
- **School trips:** speak to the trip leader confidentially well in advance. Teachers handle this more often than you'd think.
- **Avoid "labels":** don't refer to your child as "a bedwetter." Say "a child who wets the bed" — it's something they *have*, not who they *are*.
- **Siblings:** set a firm family rule — no teasing, ever.
- **Privacy:** don't discuss your child's wetting with relatives or friends without their permission.

Building independence

Children aged 6–8 can start taking some responsibility — as part of being on the team, not as a penalty:

- Placing wet pyjamas in the laundry basket.
- Helping to strip the bed.
- Getting a fresh pair of pyjamas from their night kit.

- Filling in their own chart.

✅ Clear Action Plan: Ages 6–8

Week 0: Prepare

- ☐ Have a warm, blame-free conversation with your child; share any family history.
- ☐ Record a 2-day bladder diary and 2 weeks of wet/dry nights (Appendix A).
- ☐ Book a doctor's appointment; bring your records and questions (Appendix E).

Weeks 1–4: Daytime foundations

- ☐ 6–8 drinks/day, mostly before 5 p.m.; stop 1 hour before bed; no caffeine.
- ☐ Toilet every 2–3 hours plus a double void at bedtime.
- ☐ Treat constipation if present.
- ☐ Ask school for easy access to water and the toilet.
- ☐ Protect the bed (double-make) and set up a night kit.

From week 2–4: Active treatment (with your doctor)

- ☐ **Alarm** (first choice for most): let your child help choose; practise in the day; use every night; parent wakes the child at first; record progress; continue until 14 dry nights in a row.
- ☐ Review at 4 weeks — continue if improving; see your doctor if not.
- ☐ **Or desmopressin** if the alarm isn't suitable — follow fluid rules strictly.

Always

- [ ] Reward actions, not dry nights.
- [ ] No punishment, shame, or teasing.
- [ ] Plan sleepovers and trips rather than avoiding them (Chapter 10).
- [ ] See a doctor promptly for any red flags (Chapter 3).

Chapter 7: Ages 9–12: Taking Action Before the Teen Years

What's happening at this age

Around **5% of 10-year-olds** still wet the bed — still one or two children in most classes. By this age, bedwetting is much less likely to resolve quickly on its own, especially in children who wet most nights, and its emotional impact tends to grow. Research has linked bedwetting that continues past age 10 with lower self-esteem and more emotional and behavioural difficulties in early adolescence — though these usually *improve* when the wetting is successfully treated.

This is the age of **residential school trips, camps, sleepovers, and sports tournaments**, and the age when children become intensely sensitive about fitting in. The message for parents is clear: **if your child is still wetting at 9–12, now is the time to treat it actively and persistently.** Don't wait for the teenage years.

The other big shift: your child can now be a genuine **partner and leader** in their own treatment.

The goals for ages 9–12

1. **Get a proper medical assessment** — and a referral to a specialist continence service if first-line treatments haven't worked.
2. **Put your child in the driver's seat**, with your support.

3. **Use the alarm and/or desmopressin consistently** — and combinations if needed.
4. **Plan for social events** so bedwetting never stops your child from taking part.
5. **Watch and protect emotional wellbeing.**

Step 1: Hand over ownership (with backup)

Children this age respond best when they are treated as the expert on their own body.

- **Explain the science** properly: vasopressin (the "night-time hormone"), bladder capacity, and the bladder-brain wake-up signal (Chapter 1). Many children are relieved to learn there's a physical explanation.
- **Let them read parts of this book** — Chapters 1 and 2 are written so a 10–12-year-old can follow them.
- **Agree a plan together** and write it down. Ask: "What do *you* want to try? What would help most?"
- **Give them control** over their chart, their water-bottle target, and their night kit.
- **Stay involved:** your role shifts from "doing" to "supporting" — especially with the alarm, where they may still need you to wake them.

Step 2: See the doctor (again, if needed)

If your child has never been assessed, book an appointment now, bringing a bladder diary and wet-night record (Appendix A). If your child has *already* tried treatment:

- Ask what went wrong last time. Was the alarm used every night for long enough (at least 6–8 weeks)? Was constipation

treated? Was the dose of desmopressin optimised and taken correctly?

- Ask about **combination treatment** or a **referral to a specialist** (pediatric continence clinic, pediatric urologist, or nephrologist) if two good attempts at first-line treatment have failed. NICE recommends referral when children don't respond to two complete courses of either alarm or desmopressin.
- Raise any **daytime symptoms, constipation, or snoring** again — these are common hidden reasons for treatment failure.

Step 3: Daytime foundations

The basics still matter enormously:

- **Drinks:** about 1,200–2,100 mL for girls and 1,400–2,300 mL for boys (roughly 6–10 cups), spread through the day, most before about 5 p.m. Pre-teens often drink very little at school — **this is a common cause of poor results**. Agree a specific school-day target (e.g., two full bottles).
- **Cut caffeine:** many pre-teens start drinking cola, iced tea, or energy drinks. These can significantly worsen bedwetting. Energy drinks should be avoided entirely.
- **Toilet routine:** on waking, at school breaks, after school, before dinner, and twice at bedtime.
- **Bowels:** check for constipation honestly — pre-teens may be embarrassed to discuss it. Use the Bristol Stool Chart as a neutral tool.
- **Screens and sleep:** a regular bedtime and screens off 30–60 minutes before sleep support healthy sleep patterns.

Step 4: Active treatment

The alarm

The alarm remains the best long-term option for many children at this age.

- **Choose a model your child likes** — many pre-teens prefer discreet wireless or vibrating alarms.
- **Your child should be the one to respond** to the alarm whenever possible: get up, switch it off, finish at the toilet, change, reset. If they sleep through it, a parent still needs to wake them, and that's okay.
- **Track progress** using the alarm log (Appendix C). Many 9–12-year-olds love seeing the data improve.
- **Commit:** nightly use, review after 4 weeks, continue until **14 dry nights in a row**. Consider "overlearning" (Chapter 2) to reduce relapse.

Desmopressin

Desmopressin is often extremely useful at this age because of:

- **School trips and camps**: it can be started a few weeks before an event to check it works and find the right dose, then used during the trip.
- **High night-time urine output**: common in children who wet large volumes early in the night.
- **Families who can't manage an alarm** at this stage.

Your child should understand and follow the **fluid rules** themselves — no drinks from one hour before the dose until the next morning — and know to tell an adult if they get a headache or feel sick. Remember to stop it during vomiting or diarrhoea illness.

Combination and specialist options

If first-line treatments don't work, specialists may use alarm plus desmopressin, add an anticholinergic medicine when an overactive bladder is suspected, or arrange further tests. Don't give up — the vast majority of children eventually become dry.

Step 5: Social life — plan, don't avoid

Never let bedwetting become a reason to miss out. See Chapter 10 for the full sleepover and school-trip plan. Key points:

- **Trial desmopressin** at home several weeks before a residential trip.
- **Discreet absorbent pants** designed for older children are slim and look like regular underwear.
- **Talk confidentially to trip staff** in advance; ask how they'll handle it discreetly (e.g., a teacher quietly waking your child to use the toilet, a private place to change, a bag for wet items).
- **Practice runs:** a sleepover at a relative's house can build confidence.
- **Agree a cover story** with your child, such as keeping their night pants in a toilet bag and changing in the bathroom.

Step 6: Emotional wellbeing

Watch for signs that bedwetting is weighing heavily:

- Withdrawal, refusal of invitations, avoiding friends.
- Low mood, irritability, tearfulness, or saying "I'm stupid," "I'm a baby."
- Anxiety at bedtime or trouble getting to sleep.

- Being teased or bullied.

If you see these signs, talk to your child and to your doctor — counselling or psychological support can help alongside treatment. See Chapter 11.

✅ Clear Action Plan: Ages 9–12

This week

- ☐ Have an open, blame-free conversation; explain the science; share this book's Chapters 1–2.
- ☐ Agree a written plan *together* — let your child lead.
- ☐ Start a 2-day bladder diary + 2-week wet/dry record.
- ☐ Book a doctor's appointment (or follow-up if past treatment failed).

Daily habits

- ☐ Water-bottle target at school; 6–10 drinks/day, mostly before 5 p.m.
- ☐ Zero caffeine and energy drinks.
- ☐ Toilet every 2–3 hours; double void at bedtime.
- ☐ Check for and treat constipation.
- ☐ Regular bedtime; screens off before sleep.

Treatment (with your doctor)

- ☐ Alarm used every night; review at 4 weeks; continue to 14 dry nights; consider overlearning.
- ☐ Desmopressin for trips, high night-time urine, or when an alarm isn't suitable — child knows the fluid rules.

- [ ] Ask about combinations or specialist referral if two full courses haven’t worked.

Social & emotional

- [ ] Plan every trip and sleepover (trial desmopressin early, discreet pants, confidential talk with staff).
- [ ] Watch for low mood, avoidance, or bullying — seek support early.

Chapter 8: Ages 13–16: Supporting Teens with Privacy and Respect

> **For teens reading this chapter:** This chapter is written for you as much as for your parents. You're not alone — around 1–3 in every 100 teenagers still wet the bed, which means at a school of 1,000 students, there could be 10–30 others going through the same thing. It is a medical condition with physical causes, not something you're doing wrong. And it can be treated.

What's happening at this age

By the teenage years, bedwetting is **less common, but often more persistent and more distressing**. Teens who still wet usually do so for physical reasons: high night-time urine production, a bladder that holds less overnight, difficulty waking — or a combination. Studies suggest that teenage bedwetting is more likely to be frequent and less likely to stop on its own than childhood bedwetting, so **active treatment is strongly recommended**.

Puberty brings growth spurts and hormonal changes. In some teens, this coincides with improvement; in others, it doesn't. Waiting and hoping is not a plan.

Teenagers also face unique pressures: intense need for privacy, fear of friends finding out, relationships, overnight trips, sports camps, and growing independence. Bedwetting can affect self-image, mood, sleep, and social life. Teens with bedwetting are more likely to report feeling ashamed, avoiding social events, and feeling "different" —

but successful treatment is associated with real improvements in self-esteem.

The goals for ages 13–16

1. **Respect privacy and autonomy** — the teen leads, the parent supports.
2. **Get a thorough medical assessment**, including checks for less common causes.
3. **Use effective treatment** — often desmopressin, the alarm, or both — and specialist help if needed.
4. **Support a full social life.**
5. **Protect mental health.**

For parents: shifting your role

- **Ask, don't tell.** "How are you feeling about the wetting? Do you want help with it? What would help?"
- **Respect privacy absolutely.** Never mention it in front of siblings, relatives, or friends. Knock before entering. Keep supplies discreet.
- **Offer, don't push**, a doctor's visit. Many teens welcome it once they know it's treatable. Offer that they can see the doctor alone for part or all of the visit.
- **Make practicalities invisible.** Stock supplies without comment; let your teen manage their own laundry if they prefer; provide a lidded laundry bin or wash bag in their room.
- **Stay calm and positive** about setbacks.
- **Watch for signs of distress**: withdrawal, refusing invitations, low mood, anxiety about sleeping away, or difficulty sleeping.

For teens: your action steps

1. Understand your body

Read Chapters 1 and 2 of this book. Knowing *why* it happens — night-time hormone levels, bladder capacity, and waking signals — makes it easier to fix and helps you see it's not your fault.

2. See a doctor

You can ask your parent to book the appointment, or (depending on where you live and your age) book it yourself. You have the right to speak to the doctor privately. Before you go:

- **Keep a 2-day bladder diary** (Appendix A) — what you drink, when you pee and how much, and any accidents.
- **Record wet and dry nights for 2 weeks.**
- **Note other symptoms**: needing to pee urgently or often in the day, daytime leaks, constipation, snoring, excessive thirst, weight changes.
- **Write down your questions** (Appendix E).

The doctor should test your urine and examine you. For teenagers, doctors are more likely to look carefully for underlying causes, especially if wetting has started again after being dry, if there are daytime symptoms, or if treatment hasn't worked before. They may refer you to a specialist.

3. Fix the daytime basics

These make a real difference, especially in teens:

- **Drink well during the day:** around 1,400–2,500 mL for girls and 2,100–3,200 mL for boys (roughly 6–13 cups, more if

you're very active or it's hot). Carry a water bottle and drink most of it before late afternoon.

- **Stop drinks about an hour before bed.** If you're thirsty late in the evening, it's usually a sign you didn't drink enough during the day.
- **Cut out caffeine and energy drinks**, especially after midday: coffee, cola, energy drinks, pre-workout drinks, and strong tea all increase urine production and irritate the bladder.
- **Avoid alcohol.** Alcohol strongly increases night-time urine and makes waking harder (and is illegal for under-18s in many countries).
- **Go to the toilet regularly** during the day — don't "hold it" all day at school.
- **Keep your bowels regular** — constipation is common in teens (especially with fast food, low fibre, and avoiding school toilets) and directly affects the bladder.
- **Sleep well:** regular bedtimes and phone/screens off before sleep. Very late nights and sleep deprivation make the brain harder to wake.
- **Pee twice before bed** — once when you start getting ready and again just before you fall asleep.

4. Choose a treatment with your doctor

Desmopressin is often a good fit for teens:

- It works fast and is easy to take discreetly (tablets or melts).
- It's useful for overnight trips, camps, and staying at a partner's or friend's house.
- **Follow the fluid rules strictly**: no more than small sips from 1 hour before taking it until 8 hours after. This is especially important at parties, sports events, or in hot weather. **Never take it after drinking alcohol or a lot of fluid.**

- Skip it if you're vomiting or have diarrhoea, and get medical help if you develop a headache, nausea, or confusion while taking it.

The bedwetting alarm is still highly effective for teens and is often a longer-lasting fix:

- **Vibrating or wireless alarms** are more discreet and won't wake the whole house.
- You're more likely to wake up to it yourself than a younger child — but if not, decide with your parent whether they'll help wake you during the first weeks.
- Use it every night for at least 6–8 weeks; continue until 14 dry nights in a row.

Combination or specialist treatment: if one treatment doesn't work, don't give up. Alarm plus desmopressin, adding a bladder-relaxing medicine, or specialist bladder training may be recommended. Ask for a referral to a specialist continence clinic if two full treatment courses haven't helped.

5. Manage the practical side with confidence

- **Discreet protection:** modern teen/young-adult absorbent underwear is slim and looks like regular underwear. Washable protective pants are another option.
- **Mattress protection:** a quiet, breathable waterproof protector won't crinkle.
- **Your own night kit:** spare underwear, pyjamas, wipes, a towel, and a sealable wash bag — kept somewhere private.
- **Take control of laundry** if it helps your privacy — rinse and wash wet items in the morning.
- **Shower in the morning** to stay fresh and confident.

6. Sleepovers, camps, and trips

See Chapter 10 for a full plan. Key moves for teens:

- **Trial desmopressin** at home before the event.
- **Pack discreetly:** night pants in a toiletries bag; change in the bathroom.
- **Sleeping bag strategy:** a sleeping bag with a waterproof liner or an absorbent pad inside can hide accidents.
- **Tell one trusted adult** (a coach, teacher, or camp nurse) — they handle this confidentially and more often than you'd imagine.
- **Decide what you'll say** if anything happens: you don't owe anyone an explanation. "I spilled my water" is a classic for a reason.

7. Look after your mental health

It's normal to feel embarrassed, frustrated, or fed up. But if you notice you're:

- avoiding friends, sports, or trips you used to enjoy,
- feeling down, anxious, or hopeless most days,
- being teased or bullied,
- having thoughts of hurting yourself,

please talk to someone you trust — a parent, doctor, school counsellor, or a mental health helpline. **If you ever have thoughts of harming yourself, seek help immediately** from emergency services or a crisis line in your country.

✅ Clear Action Plan: Ages 13–16

For the teen

- ☐ Read Chapters 1–2 to understand your body.
- ☐ Keep a 2-day bladder diary and 2 weeks of wet/dry nights.
- ☐ See a doctor (you can ask to talk privately); bring your records and questions.
- ☐ Drink well during the day; stop 1 hour before bed; no caffeine, energy drinks, or alcohol.
- ☐ Pee regularly through the day and twice before bed; keep bowels regular.
- ☐ Choose a treatment with your doctor: desmopressin (follow fluid rules strictly), vibrating/wireless alarm, or both.
- ☐ Set up a private night kit and quiet bed protection.
- ☐ Plan every trip: trial medication early, pack discreetly, tell one trusted adult.
- ☐ Talk to someone if your mood is suffering.

For the parent

- ☐ Ask, don’t tell; let your teen lead.
- ☐ Guarantee privacy — at home, with relatives, and with siblings.
- ☐ Offer a doctor’s appointment and private consultation time.
- ☐ Supply discreet protection and a private laundry option without comment.
- ☐ Watch for signs of low mood or social withdrawal, and get support early.
- ☐ Push for specialist referral if two treatment courses haven’t worked.

Chapter 9: Ages 16–19: Moving Toward Independence

> **For young people reading this chapter:** If you're 16–19 and still wetting at night, you might feel like you're the only one. You're not. Studies estimate that around 1–2% of older teens and adults experience bedwetting — that's millions of people worldwide. It's a medical condition, it can be investigated and treated, and you deserve good care for it.

What's happening at this age

By the late teens, **bedwetting that has persisted since childhood is less likely to resolve on its own** without treatment, and wetting that starts *new* at this age (secondary enuresis) always needs a medical check. Young people at this stage are also going through major life changes: finishing school, jobs, driving, first relationships, travel, college or university, and possibly moving out of home into shared accommodation. These transitions can make bedwetting feel especially daunting — but they also make it a perfect time to take decisive action.

The goals for ages 16–19

1. **Take ownership** of your own health care.
2. **Get a thorough assessment** — ideally including specialist review if earlier treatment has failed.
3. **Find a treatment strategy that fits adult life.**

4. **Plan confidently** for college, travel, relationships, and independent living.
5. **Protect your mental wellbeing.**

Step 1: Take charge of your medical care

At this age, you can usually book and attend appointments on your own and speak to a doctor confidentially.

- **Book an appointment** with your family doctor or primary care provider. Say clearly: “I’ve had bedwetting [since childhood / since a certain time] and I’d like help with it.”
- **Bring data**: a 2–3 day bladder diary (Appendix A), 2 weeks of wet/dry nights, and a list of previous treatments and how they went.
- **Mention any other symptoms**: daytime urgency or leaks, frequent peeing, weak stream, constipation, snoring or breathing pauses, excessive thirst, weight loss, back pain or leg symptoms, anxiety or low mood.
- **Expect a thorough check**: urine test, examination, and possibly further tests such as blood tests, a kidney and bladder ultrasound, a test measuring urine flow and how well the bladder empties, or a sleep study if sleep-disordered breathing is suspected. Persistent bedwetting at this age warrants a careful look for treatable causes such as an overactive bladder, a bladder that doesn’t empty fully, or obstructed breathing during sleep.
- **Ask for a specialist referral** (urologist, nephrologist, or specialist continence service) if first-line treatments haven’t worked.

Step 2: Plan for the transition from pediatric to adult care

If you've been seen by a children's specialist, ask them about **transition**: being handed over to an adult service in good time, rather than being discharged with nowhere to go. Ask for:

- A written summary of your diagnosis, tests, and treatments so far.
- A named adult service or contact.
- Your current prescriptions and how to renew them.

Step 3: Lifestyle factors that matter more at this age

- **Fluids:** aim for around 1,400–2,500 mL (women) or 2,100–3,200 mL (men) through the day, most before late afternoon. Stop drinking about an hour before bed.
- **Caffeine:** coffee, energy drinks, and pre-workout supplements are common at this age and can substantially worsen bedwetting — avoid them after midday or altogether.
- **Alcohol:** alcohol suppresses the hormone that reduces night-time urine, increases urine volume, and makes you harder to wake. It's one of the biggest triggers for wetting in young adults. If you drink (and are of legal age), limit it, avoid it on nights you want to stay dry, and **never combine it with desmopressin**.
- **Cannabis and other substances** can deepen sleep and affect bladder control — another reason to avoid them.
- **Sleep:** irregular schedules, all-nighters, and sleep debt make the brain harder to wake. Keep a reasonably regular sleep pattern.

- **Bowels:** student diets low in fibre and high in fast food lead to constipation. Keep fibre and fluids up.
- **Weight and exercise:** regular physical activity supports bowel and bladder health. Pelvic floor exercises taught by a physiotherapist may help some people with daytime symptoms.

Step 4: Treatment options for young adults

- **Desmopressin:** often the most practical option for independent life, travel, and shared living. It requires disciplined fluid rules (no drinks from 1 hour before to 8 hours after) and **must not be taken after drinking alcohol or large volumes of fluid**. Note that safety precautions are similar for adults; your doctor will review the dose and formulation.
- **Bedwetting alarm:** still effective at this age. Vibrating or wireless alarms suit shared housing and dorms. If you live alone and sleep through alarms, a pad alarm with a loud bedside unit, a vibrating pillow pad, or a light plus sound combination may help.
- **Combination therapy** (desmopressin + alarm, or desmopressin + a bladder-relaxing medicine) when one treatment isn't enough.
- **Specialist treatments** for an overactive bladder or incomplete bladder emptying, if these are found.
- **Treatment of sleep-disordered breathing**, if identified.
- **Psychological support** — especially cognitive behavioural therapy (CBT) — to manage anxiety, shame, and the impact on your life, alongside medical treatment.

Step 5: Living with confidence

College, university, and shared housing

- **Request a single room** if possible. Many colleges will accommodate medical needs confidentially — a letter from your doctor may help. You don't have to disclose the specific condition to housing staff in detail.
- **Disability or student support services** can arrange accommodations confidentially.
- **Set up your room:** quiet waterproof mattress protector, layered bedding, a lidded laundry bag, and a discreet supply of protection.
- **Laundry plan:** a portable laundry bag, a schedule that suits you, and spare sheets.

Travel and holidays

- **Pack a travel kit:** absorbent underwear, a portable waterproof sheet or travel bed pad, disposable bags, wipes, a change of clothes.
- **Carry your medication in hand luggage** with a copy of the prescription.
- **Time zones:** take desmopressin at your local bedtime and follow fluid rules; discuss long-haul travel with your doctor.
- **Hotels and hostels:** a waterproof travel sheet protects the mattress and avoids awkward conversations.

Relationships

- **You decide** if, when, and how to tell a partner. There's no obligation to disclose early in a relationship.

- Many people find that when they do share it, partners are understanding — especially when it's explained as a medical condition being treated.
- A simple script: "I want to tell you something personal. I have a medical condition that sometimes causes me to wet the bed. It's being treated, and I manage it with [protection/medication]. I wanted you to know."
- Plan practicalities for overnight stays: desmopressin, discreet protection, or a waterproof pad in your bag.

Work

- Bedwetting rarely affects work directly, but shift work, travel, and work trips can raise practical issues. Use the same planning approach as for trips.

Step 6: Protecting your mental health

Living with bedwetting as a young adult can bring shame, anxiety around intimacy and travel, and avoidance of opportunities. You deserve support:

- Talk to your doctor about how it's affecting you — psychological impact is a valid reason to escalate treatment.
- Consider counselling or CBT.
- Connect with **reputable continence charities and patient organisations** in your country — many run helplines and online information for young people.
- **If you're feeling hopeless or having thoughts of self-harm**, contact emergency services or a crisis line in your country immediately.

✅ Clear Action Plan: Ages 16–19

Medical

- ☐ Book your own appointment; bring a bladder diary, wet/dry record, and treatment history.
- ☐ Report all symptoms, including daytime bladder symptoms, constipation, snoring, thirst, and mood.
- ☐ Ask about further tests and specialist referral if earlier treatments haven't worked.
- ☐ If you're under a children's specialist, request a planned transition to adult services and a written summary.

Lifestyle

- ☐ Drink well by day; stop 1 hour before bed.
- ☐ Avoid caffeine/energy drinks after midday; limit alcohol and never mix it with desmopressin.
- ☐ Keep a regular sleep schedule and healthy bowels.

Treatment

- ☐ Choose a plan with your doctor: desmopressin, alarm (vibrating/wireless), combination, or specialist therapy.
- ☐ Follow desmopressin fluid rules strictly; know the warning signs of low sodium.

Independent life

- ☐ Arrange a single room or confidential accommodation at college if needed.
- ☐ Build a travel kit and a laundry routine.
- ☐ Decide on your own terms if and how to tell a partner.

- [ ] Seek support for anxiety, low mood, or shame — CBT and counselling help.

Part Three: Living Well While Getting Dry

Chapter 10: Sleepovers, Camps, Travel, and Setbacks

Bedwetting should never be the reason a child misses out. With planning, almost every child can attend sleepovers, camps, and trips — and doing so is important for their confidence and friendships.

The sleepover plan

Before the sleepover

1. **Talk with your child** about whether they want to go and what would help them feel confident.
2. **Choose the right protection:**
    - Discreet absorbent pants (slim, age-appropriate designs look like underwear).
    - A sleeping bag with a waterproof liner or an absorbent pad tucked inside.
    - Dark-coloured pyjamas.
3. **Consider desmopressin** if prescribed — try it at home first, ideally 2–3 weeks before, to check it works and the dose is right.
4. **Decide whether to tell the host parent.** For younger children, a quiet word with the host parent usually helps — most are understanding, and many have dealt with it themselves. Ask them to be discreet. For older children, respect their wishes.
5. **Pack a discreet kit:** night pants in a toiletries bag or pencil case, a plastic bag for used items, wipes, spare underwear.
6. **Plan the routine:** change into night pants in the bathroom; go to the toilet before sleeping; set a phone vibration alarm for a

middle-of-the-night toilet trip if it helps.

7. **Rehearse an exit line:** “I spilled my drink” if anything gets wet.

On the night

- Drink well earlier in the day; limit drinks in the evening (strictly, if taking desmopressin). Watch the sleepover snacks — fizzy drinks and cola are common.
- Go to the toilet before bed.

Afterwards

- Praise your child for going, whatever happened.
- If there was an accident, treat it as a normal hiccup and plan together for next time.

Start small: a sleepover at a grandparent’s or cousin’s house, or a “late-over” (staying until bedtime and coming home), builds confidence.

Camps and residential school trips

- **Contact the trip leader or camp nurse confidentially** at least 2–3 weeks before. Experienced staff manage bedwetting regularly and discreetly.
- **Agree a plan with staff**, which might include:
    - a staff member quietly waking your child for a toilet trip before staff go to bed;
    - giving desmopressin at bedtime (supply the medication with clear written instructions);
    - a discreet place to change and store supplies;
    - handling wet items quietly in a sealed bag;
    - a bottom bunk (easier and quicker to get to the toilet).

- **Trial desmopressin at home** for several weeks before. Discuss timing and dose with your doctor.
- **Pack:** absorbent pants, sleeping bag liner, plastic bags, wipes, dark pyjamas, extra underwear.
- **Reassure your child** that staff are on their side.

Family holidays and travel

- **Pack a travel kit:** portable waterproof mattress protector or travel bed pads, absorbent pants, wipes, bags, spare pyjamas.
- **Carry medication** in hand luggage with a copy of the prescription.
- **Plan for heat:** hot climates increase thirst — while taking desmopressin, be careful about large evening drinks; discuss with your doctor.
- **Alarm treatment and holidays:** you can pause alarm treatment during a short holiday and restart afterwards. For a long trip, consider postponing the start of an alarm program until you're home.
- **Find the toilet** in a new place before bedtime and leave a light on.

Handling setbacks and relapse

Progress is rarely a straight line. Wet nights can return during:

- illness, especially infections or anything causing fever;
- stress — exams, moving house, family changes, bullying;
- changes in routine — holidays, late nights, extra drinks, parties;
- growth spurts;
- stopping an alarm or medication.

What to do:

1. **Stay calm.** A setback is not failure. Tell your child: “This happens. Your body still knows how to do it — we’ll get back on track.”
2. **Check the basics:** drinks, caffeine, toilet habits, constipation, sleep.
3. **Look for triggers** and red flags. If wetting returns after six months of dryness, or comes with other symptoms, see a doctor.
4. **Restart treatment:** if the alarm worked before, restarting it usually works faster the second time. If desmopressin was stopped, your doctor may restart it and plan a more gradual reduction.
5. **Review with your doctor** if relapses keep happening.

Chapter 11: Emotional Wellbeing, Family Life, and Communication

Bedwetting is a physical condition — but it is lived emotionally, by the child and the whole family.

How bedwetting can affect a child

Children and teenagers who wet the bed commonly describe:

- embarrassment and shame;
- fear of others finding out;
- avoiding sleepovers, camps, and trips;
- feeling “different” or “babyish”;
- worrying at bedtime;
- frustration and disappointment after wet nights;
- in some cases, teasing or bullying.

Research suggests bedwetting can affect self-esteem, particularly in older children — and that **successful treatment improves self-esteem**. This is one of the strongest reasons to treat rather than wait.

The parent’s emotional load

Parents’ feelings matter too. Night-time disruption, extra laundry, and worry are exhausting. It’s normal to feel frustrated — but it’s important that frustration is never directed at the child. Parental intolerance of wetting is known to be associated with poorer treatment outcomes and can harm the child emotionally.

Look after yourself:

- Share the night-time duties with a partner or other adult where possible.
- Simplify the work (double-made beds, night kits, absorbent pads).
- Remind yourself: your child is not doing this on purpose — and it will get better.
- If you're feeling overwhelmed, tell your doctor. NICE guidance specifically recognises that parental stress can affect which treatment is suitable, and support is available.

What to say (and not say)

Instead of…	Try…
"Not again!"	"No problem, let's get you changed."
"You're too old for this."	"Lots of kids your age deal with this. We'll figure it out together."
"Why can't you just wake up?"	"Your brain is still learning to wake up to your bladder. We're going to help it learn."
"If you're dry, you'll get a toy."	"If you drink your water and go to the toilet before bed, you get a star."
"Your brother was dry at 4."	"Every body develops on its own timetable."
"Don't tell anyone."	"This is private. You get to decide who knows."

Siblings

- **Zero tolerance for teasing.** Make this an explicit family rule.
- **Explain it to siblings** in simple terms: "It's something their body is still learning. It's not their fault."

- **Protect privacy** — siblings shouldn't share it with friends.
- **Be fair**: rewards for the child who wets should be about effort, and siblings can have their own chart for something they're working on.
- **Shared bedrooms**: consider a vibrating alarm or a wireless alarm with the receiver in a parent's room.

Bullying

If your child is being teased or bullied about bedwetting:

1. Listen and reassure them it's not their fault.
2. Contact the school; most schools have anti-bullying policies and will act discreetly.
3. Help your child plan responses (e.g., walking away, telling an adult).
4. Consider counselling support if it's affecting their wellbeing.

When to seek psychological support

Consider a referral to a counsellor, psychologist, or child mental health service if your child:

- shows persistent low mood, anxiety, or withdrawal;
- avoids school or social activities;
- shows signs of trauma or significant stress (especially if wetting started suddenly after a difficult event);
- has behaviour difficulties that make treatment hard;
- expresses thoughts of self-harm (seek help urgently).

Psychological support works best *alongside* medical treatment, not instead of it.

Chapter 12: Special Situations and Practical Care

Children with ADHD, autism, or developmental differences

Bedwetting is more common in children with ADHD and autism, and in children with developmental delay. They can still be treated successfully, but approaches may need adapting:

- **ADHD:** Children may forget to drink or go to the toilet during the day, and may find alarm routines harder to follow. Use visual timers, phone reminders, and very simple routines. Some ADHD medications or their timing may affect sleep and appetite — discuss with your doctor. Constipation is also more common.
- **Autism:** Sensory sensitivities can make wearable alarms, new pyjamas, or certain bedding uncomfortable. Mat alarms or vibrating alarms may suit better. Use visual schedules and social stories to explain the plan. Introduce changes gradually. Watch for toilet avoidance due to sensory issues (noisy hand dryers, unfamiliar toilets).
- **Learning disabilities and developmental delay:** Readiness depends on developmental age as much as actual age. Specialist continence services can provide tailored toileting programs.

In all cases, ask your doctor about referral to a specialist continence service experienced in neurodevelopmental conditions.

Children with diabetes, kidney conditions, or other medical conditions

- **Diabetes:** new or worsening wetting in a child with diabetes may signal high blood sugar — check blood glucose and contact your diabetes team.
- **Kidney or urological conditions, spina bifida, or other neurological conditions:** bedwetting management should be led by the specialist team.
- **Sickle cell disease:** these children often produce more urine and may wet more often; their care team can advise.
- **Sleep apnea:** treating obstructed breathing (sometimes by removing enlarged tonsils and adenoids) can improve or resolve bedwetting.

When stress or trauma may be involved

Sudden-onset (secondary) wetting can follow stressful events such as bereavement, family breakdown, bullying, or moving home. Rarely, it may be a sign of abuse. If your child's wetting started suddenly together with other changes — fearfulness, withdrawal, sexualised behaviour, unexplained injuries, or reluctance to be with a particular person — talk to your doctor or child protection services. Professionals are trained to handle these concerns sensitively.

Practical care: cleaning up and skin care

Skin care

- Wash or shower your child each morning after a wet night to prevent rashes and odour.
- Use mild, fragrance-free soap and pat dry.

- A barrier cream can help if skin becomes red or sore.
- See a doctor if a rash doesn't improve or if there's pain.

Laundry tips

- **Rinse urine-soaked items in cold water** first, then wash as normal. Hot water can set urine odour and stains.
- **Add white vinegar** (about half a cup / 120 mL) to the rinse cycle or **bicarbonate of soda / baking soda** to the wash to neutralise odour. Enzyme-based laundry products also help.
- **Dry in sunlight** where possible — it's a natural deodoriser and stain remover.
- **Have at least three sets** of sheets and mattress protectors in rotation.
- **Use a lidded laundry bin** to contain odour.

Mattress care

- Always use a waterproof mattress protector.
- If a mattress gets wet: blot (don't rub) as much as possible, sprinkle with baking soda, leave for several hours, then vacuum. An enzyme cleaner can help with odour. Allow to dry completely.

Absorbent products

- Choose the right size — products that are too big or too small leak.
- Try a few types: disposable pull-ups, washable/reusable night pants, and bed pads.
- For older children and teens, choose discreet youth/adult-sized designs.
- Remember that for alarm treatment, absorbent pants usually need to be removed.

Keeping costs down

- Ask your doctor whether **alarms can be loaned** through a local continence service or health system.
- Washable bed pads and night pants can be more economical than disposables over time.
- Check whether your health insurance or national health service covers alarms, medication, or continence products.

Final words: Keep going

Bedwetting can feel like it will last forever. It won't. Nearly every child eventually becomes dry, and with the right approach, most can get there much sooner than by waiting alone.

Remember the four pillars of this book:

1. **Understand** — it's physical, common, and nobody's fault.
2. **Build the foundations** — good drinking, regular toileting, healthy bowels, and healthy sleep.
3. **Treat actively** — alarm, desmopressin, or both, with your doctor's support, at the right age for your child.
4. **Protect confidence** — kindness, privacy, planning, and never letting wet nights stop your child from living fully.

Your patience, calm, and belief in your child are the most powerful tools in this book. Dry nights are ahead.

Appendices: Printable Tools

Appendix A: Bladder Diary and Wet-Night Record

Part 1 — Two-day daytime bladder diary. Choose two days (a weekend is easiest). Use a measuring jug to measure each pee, and note every drink.

Time	Drink (type & amount in mL)	Pee amount (mL)	Wet/leak? (Y/N)	Urgent? (Y/N)	Notes

Tip: The largest single pee (excluding the first pee in the morning) gives your doctor an idea of bladder capacity. A rough expected capacity for children is (age + 1) × 30 mL — for example, about 210 mL for a 6-year-old.

Part 2 — Two-week wet-night record. To estimate night-time urine production, you can weigh the night pants or pad in the morning (weight of wet pad minus dry pad, in grams ≈ mL) and add the first morning pee.

Night	Date	Dry/Wet	Number of wettings	Size of patch (S/M/L)	Woke up to pee?	Morning pee (mL)	Wet pad weight (g)
1							

Night	Date	Dry/Wet	Number of wettings	Size of patch (S/M/L)	Woke up to pee?	Morning pee (mL)	Wet pad weight (g)
2							
3							
4							
5							
6							
7							
8							
9							
10							
11							
12							
13							
14							

Part 3 — One-week bowel record.

Day	Poo? (Y/N)	Stool type (Bristol 1–7)	Pain or straining?	Soiling?
Mon				
Tue				
Wed				
Thu				
Fri				
Sat				

Day	Poo? (Y/N)	Stool type (Bristol 1–7)	Pain or straining?	Soiling?
Sun				

Bristol Stool Chart in brief: Types 1–2 are hard lumps (suggesting constipation); types 3–4 are smooth and soft (ideal); types 5–7 are soft blobs to watery (may suggest diarrhoea or overflow from constipation).

Appendix B: Reward Chart (Actions, Not Dry Nights)

Child's name: _____ Week starting: _____

Action	Mon	Tue	Wed	Thu	Fri	Sat	Sun
Drank my water bottle(s)							
Went to the toilet before bed (twice!)							
No drinks in the last hour before bed							
Got up to the toilet / responded to the alarm							
Helped with sheets or pyjamas							
Stars today							

Our agreed reward for ____ stars: _____

Appendix C: Bedwetting Alarm Progress Log

Start date: _____ Alarm type: _____ Review date (4 weeks): _____

Night	Dry?	Alarm went off (time)	Woke on own?	Parent woke child?	Finished at toilet?	Patch size (S/M/L)	Dry-night streak
1							
2							
3							
4							
5							
6							
7							
8							
9							
10							
11							
12							
13							
14							

(Copy this page for weeks 3–16.) **Signs of progress:** smaller wet patches, the alarm going off later in the night, fewer wettings per night, waking to the alarm without help, waking *before* the alarm, dry nights. **Goal:** 14 dry nights in a row.

Appendix D: Daily Fluid Guide and Drinking Schedule

Daily totals (water-based drinks):

Age	Girls	Boys
4–8 years	1,000–1,400 mL	1,000–1,400 mL
9–13 years	1,200–2,100 mL	1,400–2,300 mL
14–18 years	1,400–2,500 mL	2,100–3,200 mL

Sample drinking schedule (school day):

Time	Drink
On waking / breakfast	1 drink
Mid-morning break	1 drink
Lunch	1 drink
Mid-afternoon	1 drink
After school	1 drink
Evening meal	1 drink
1 hour before bed onward	Stop (small sips only if thirsty)

Drinks to avoid or limit: cola, energy drinks, coffee, tea, chocolate drinks (caffeine); fizzy drinks; sugary juices and squashes in large amounts.

Appendix E: Doctor's Visit Checklist and Questions

Bring with you:

- ☐ Two-day bladder diary

- [ ] Two-week wet-night record
- [ ] One-week bowel record
- [ ] List of other symptoms (daytime wetting, urgency, snoring, thirst, mood)
- [ ] Family history of bedwetting
- [ ] Treatments already tried, how long, and results
- [ ] Current medicines

Questions to ask:

1. What type of bedwetting does my child have (primary/secondary, with or without daytime symptoms)?
2. Is there anything that needs testing or investigating?
3. Is my child constipated? How should we treat it?
4. Would an alarm or desmopressin suit my child better — and why?
5. How long should we try the treatment before reviewing it?
6. Can we borrow an alarm through a local service?
7. If desmopressin: what are the fluid rules, when should we stop it, and what warning signs should we watch for?
8. What should we do if the treatment doesn't work?
9. When would you refer us to a specialist?
10. Is there support available for my child's feelings or for our family?

Appendix F: Quick Reference — Actions by Age

Age	Is wetting normal?	Main focus	Key actions	See a doctor if…
0–2	Yes — fully normal	Healthy foundations	Water through the day; fibre; prevent constipation; relaxed daytime potty training when ready; keep night diapers	Constipation, constant dribbling, weak stream, unexplained fevers, excessive thirst, loud snoring
3–5	Yes — common (about 1 in 5 at age 5)	Readiness and habits	Watch for dry mornings; try nights without pants when ready; 6–8 drinks/day; regular toilet visits; double void; calm responses; sticker charts for actions	Age 5+ and bothered by it; daytime symptoms; constipation; wetting after 6 months dry; red flags
6–8	Common (7–10%) but now treatable	Active treatment begins	Doctor's assessment; optimise drinks/toilet/bowels; **alarm** first-line (or desmopressin); reward effort; plan sleepovers	Treatment recommended — book an assessment
9–12	Less common (about 5%)	Persistent, child-led treatment	Child leads the plan; school water target; no caffeine; alarm and/or desmopressin; trial medication before trips; watch mood	Not responding after 2 treatment courses → specialist

Age	Is wetting normal?	Main focus	Key actions	See a doctor if…
13–16	Uncommon (1–3%)	Privacy, autonomy, effective treatment	Private consultations; strict caffeine/alcohol avoidance; desmopressin and/or discreet alarm; private night kit; trip planning; mental health	Always worth assessment; specialist if treatment fails; any red flags
16–19	Uncommon (1–2%)	Independence and adult care	Own appointments; thorough tests; transition to adult services; alcohol awareness; college/travel/relationship planning; CBT/support	Always; new-onset wetting needs prompt assessment

Glossary

Alarm (enuresis alarm): A device that sounds or vibrates when it detects moisture, used to train a child to wake when the bladder is full.

Anticholinergic: A type of medicine (e.g., oxybutynin) that relaxes an overactive bladder.

Bladder capacity: The amount of urine the bladder can comfortably hold.

Bladder diary (frequency-volume chart): A record of drinks, urine volumes, and accidents.

Bristol Stool Chart: A seven-point scale describing stool types, used to assess constipation.

Constipation: Infrequent, hard, or painful bowel movements, or incomplete emptying.

Desmopressin: A synthetic version of vasopressin that reduces night-time urine production.

Double voiding: Going to the toilet twice in quick succession (e.g., at the start and end of the bedtime routine) to empty the bladder fully.

Enuresis (nocturnal): Wetting during sleep in a child aged 5 or older.

Hyponatremia: A low level of sodium in the blood, a rare but serious risk if a person drinks too much while taking desmopressin.

Lifting: Carrying or waking a child to the toilet during the night.

Monosymptomatic enuresis: Night-time wetting without daytime bladder symptoms.

Nocturnal polyuria: Producing an unusually large amount of urine at night.

Non-monosymptomatic enuresis: Night-time wetting with daytime bladder symptoms.

Overactive bladder: A bladder that contracts suddenly and frequently, causing urgency and frequent urination.

Overlearning: Continuing alarm treatment with an extra evening drink after initial success, to reduce relapse.

Primary enuresis: Bedwetting in a child who has never been consistently dry at night.

Secondary enuresis: Bedwetting that returns after at least six months of dryness.

Vasopressin (antidiuretic hormone, ADH): A natural hormone that tells the kidneys to make less urine.

References and Further Reading

The guidance in this book is based on the following clinical guidelines, reviews, and studies. Readers and health professionals may wish to consult them directly.

1. National Institute for Health and Care Excellence (NICE). *Bedwetting in under 19s.* Clinical guideline CG111. London: NICE; 2010 (reviewed 2018). https://www.nice.org.uk/guidance/cg111
2. NICE. *Bedwetting in under 19s — Information for the public: general advice and initial treatment.* https://www.nice.org.uk/guidance/cg111/ifp/chapter/general-advice
3. Nevéus T, Fonseca E, Franco I, et al. Management and treatment of nocturnal enuresis — an updated standardization document from the International Children's Continence Society. *Journal of Pediatric Urology.* 2020;16(1):10–19.
4. Austin PF, Bauer SB, Bower W, et al. The standardization of terminology of lower urinary tract function in children and adolescents: update report from the standardization committee of the International Children's Continence Society. *Neurourology and Urodynamics.* 2016;35(4):471–481.
5. Caldwell PHY, Deshpande AV, Von Gontard A. Management of nocturnal enuresis. *BMJ.* 2013;347:f6259.
6. Glazener CMA, Evans JHC, Peto RE. Alarm interventions for nocturnal enuresis in children. *Cochrane Database of Systematic Reviews.* 2005 (and updates).
7. Glazener CMA, Evans JHC. Desmopressin for nocturnal enuresis in children. *Cochrane Database of Systematic Reviews.* 2002 (and updates).

8. Forsythe WI, Redmond A. Enuresis and spontaneous cure rate: study of 1129 enuretics. *Archives of Disease in Childhood.* 1974;49(4):259–263.
9. Bedwetting and toileting problems in children. *Medical Journal of Australia.* 2005;182(4):190–195. https://www.mja.com.au/journal/2005/182/4/4-bedwetting-and-toileting-problems-children
10. US Food and Drug Administration. Information for healthcare professionals: desmopressin acetate (marketed as DDAVP nasal spray, DDAVP rhinal tube, DDAVP, DDVP, Minirin, and Stimate nasal spray). FDA alert, December 2007.
11. Medicines and Healthcare products Regulatory Agency (MHRA). Desmopressin nasal spray: removal of nocturnal enuresis indication. *Drug Safety Update.* https://www.gov.uk/drug-safety-update/desmopressin-nasal-spray-removal-of-nocturnal-enuresis-indication
12. Ozgür BC, Ozgür S, Doğan V, Orün UA. The efficacy of an enuresis alarm in monosymptomatic nocturnal enuresis. *Singapore Medical Journal.* 2009;50(9):879–880.
13. Von Gontard A, Baeyens D, Van Hoecke E, Warzak WJ, Bachmann C. Psychological and psychiatric issues in urinary and fecal incontinence. *Journal of Urology.* 2011;185(4):1432–1436.
14. Hägglöf B, Andrén O, Bergström E, Marklund L, Wendelius M. Self-esteem in children with nocturnal enuresis and urinary incontinence: improvement of self-esteem after treatment. *European Urology.* 1998;33(Suppl 3):16–19.
15. Joinson C, Heron J, Emond A, Butler R. Psychological problems in children with bedwetting and combined (day and night) wetting: a UK population-based study. *Journal of Pediatric Psychology.* 2007;32(5):605–616.

16. Lewis SJ, Heaton KW. Stool form scale as a useful guide to intestinal transit time. *Scandinavian Journal of Gastroenterology.* 1997;32(9):920–924.
17. NHS and NHS Trust patient information on children's bladder and bowel health and fluid intake (e.g., Wrightington, Wigan and Leigh NHS Foundation Trust; Kingston and Richmond NHS Foundation Trust).
18. American Academy of Pediatrics. HealthyChildren.org — Bedwetting (enuresis) parent information. https://www.healthychildren.org

Useful organisations (check for equivalents in your country):

- **ERIC — The Children's Bowel & Bladder Charity** (UK): eric.org.uk
- **Bladder & Bowel UK**: bbuk.org.uk
- **National Association For Continence** (USA): nafc.org
- **Continence Foundation of Australia**: continence.org.au
- **International Children's Continence Society**: i-c-c-s.org
- **HealthyChildren.org** (American Academy of Pediatrics)

About the Author

[Author biography — add your background, credentials, and why you wrote this book.]

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